

EXPLANATORY STATEMENT

Health Insurance Act 1973

Health Insurance (Share by Default) Rules 2025

Purpose and operation

The *Health Insurance (Share by Default) Rules 2025* (the Instrument) sets out the scope of professional services for which medicare benefits will only be payable if information is uploaded to the My Health Record system.

The Instrument gives effect to amendments to the *Health Insurance Act 1973* (the Act) made by the *Health Legislation Amendment (Modernising My Health Record - Sharing by Default) Act 2025* (SBD Act). These reforms set out a new regulatory framework for mandating the sharing of specified health information to the My Health Record system. The Instrument specifies certain pathology and diagnostic imaging services as subject to new upload conditions in order for medicare benefits to be payable, by reference to medicare benefit scheme tables as set out in other health insurance regulations. The Instrument outlines the information required to be uploaded are approved pathology practitioner and radiologist reports resulting from the specified pathology and diagnostic imaging services. Uploading to the My Health Record system will be required within 24 hours of the report being sent to a requesting or treating practitioner. Approved pathology practitioners and radiologists must arrange uploading of the relevant information or be associated with a healthcare provider organisation that is registered for My Health Record under the *My Health Records Act 2012* (MHR Act) and processes the uploads. The upload requirement is subject to exceptions outlined in section 19AD of the Act, including when:

- a. the healthcare recipient or their representative has advised that they do not want the information uploaded
- b. a healthcare provider reasonably believes that the information should not be uploaded due to a serious concern for the health, safety or wellbeing of the healthcare recipient
- c. the healthcare recipient is not registered for My Health Record
- d. the information cannot be shared due to other reasons beyond the control of the healthcare provider.

Background

Sharing of information to an individual's My Health Record is currently voluntary for healthcare providers. The SBD Act, passed in February 2025, amended the MHR Act and the Act to implement a new framework to mandate the sharing of individuals' health information to My Health Record for the first time. The SBD Act reforms responded to recommendations in the Strengthening Medicare Taskforce Report released in 2022. The Report highlighted that access to real time health information is a critical foundation for a modern and connected health care system and recommended modernising My Health Record to significantly increase health information available to consumers and their healthcare providers by:

- requiring 'sharing by default' for private and public practitioners and services, and
- making it easier for people and their health care teams to use at the point-of-care.

The SBD Act also amended the MHR Act to create an obligation on specified constitutional corporations to become registered under the MHR Act and to upload prescribed information, regardless of whether medicare benefits are payable or not. The amendments in the SBD Act ensure that where an obligation applies to either a constitutional corporation or an individual healthcare provider or both, the obligation will be satisfied if either uploads the prescribed information. The initial scope of the proposed upload requirements are reports authorised by pathologists and radiologists as outlined in this Instrument as well as rules made under the MHR Act.

Authority

Section 19AI of the Act provides that the Minister may make rules about matters required or permitted by this Act to be prescribed by the upload rules.

Subsection 19AD(1) provides that rules may specify the types of professional services rendered by (or on behalf of) specified persons, for which medicare benefits are not payable if prescribed information is not uploaded to the My Health Record system (as defined in section 5 of the MHR Act). Subsection 19AD(1) also enables the rules to specify the period in which information must be uploaded to the My Health Record system.

Commencement

This instrument commences on 1 July 2026.

Consultation

The SBD Act and this Instrument have been developed following extensive consultation with the relevant industry sectors, medical peak bodies, consumer bodies, other Australian jurisdictions and the Australian public.

There was broad support from Australian governments and stakeholders for sharing pathology and diagnostic imaging reports by default to My Health Record.

A Clinical Reference Group was established by the Australian Digital Health Agency to provide clinical and strategic advice to support safe implementation of these reforms, including proposed exceptions and safeguards. The clinical and strategic advice and recommendations from the Clinical Reference Group will also be incorporated in the development of a Clinical Safety Guide to assist in implementing these reforms. This group has been replaced by an ongoing National Clinical Governance Committee for Digital Health. The Committee will evaluate the implementation and effectiveness of these reforms as well as providing ongoing clinical guidance.

General

This instrument is a legislative instrument for the purposes of the *Legislation Act 2003*.

Details of this instrument are set out in **Attachment A**.

This instrument is compatible with the human rights and freedoms recognised or declared under section 3 of the *Human Rights (Parliamentary Scrutiny) Act 2011*. A full statement of compatibility is set out in **Attachment B**.

Details of the *Health Insurance (Share by Default) Rules 2025*

Section 1 – Name

Section 1 provides that the name of the instrument is the *Health Insurance (Share by Default) Rules 2025*.

Section 2 – Commencement

This instrument commences on 1 July 2026.

Section 3 – Authority

Section 3 provides that the instrument is made under the *Health Insurance Act 1973* (the Act).

Section 4 – Definitions

Section 4 provides the following definitions for this instrument:

Act is defined as the *Health Insurance Act 1973*

radiologist is defined as a medical practitioner who is recognised as a specialist in diagnostic radiology:

- a. under subsection 3D(1) of the Act; or
- b. by a determination under paragraph 3DB(4)(a) or subsection 3E(1) of the Act that is in effect.

The note to section 4 also clarifies that a number of terms used in this instrument are defined in the Act including approved pathology practitioner, My Health Record system, pathologist determinable service, pathology service and R-type diagnostic imaging service.

Part 2 – Sharing health information by default

Division 1 – Diagnostic imaging services

Section 5 – Diagnostic imaging services—specified professional services and persons

Section 5 outlines the types of diagnostic imaging services and practitioners impacted by the upload condition, subject to any additional exceptions outlined in the Act or rules made under the MHR Act.

Section 5 specifies for the purpose of subsection 19AD(1), the following services:

- a. R-type diagnostic imaging services referred to in an item of the table in Part 2 of Schedule 1 to the Health Insurance (Diagnostic Imaging Services Table) Regulations (No. 2) 2020 or additionally determined by the Minister to be treated as if it were in that table; and

- b. medical services referred to in an item of the table in Part 4 of Schedule 1 to the Health Insurance (General Medical Services Table) Regulations 2021 or additionally determined by the Minister to be treated as if it were in that table.

The specified services need to be rendered by or on behalf of a radiologist to be impacted by the upload condition.

Section 6 - Diagnostic imaging services—information to be shared with the My Health Record system

Section 6 specifies the information that needs to be uploaded to the My Health Record system. It specifies for the purposes of subsection 19AD(1), the information contained in a report or other document that is created in relation to the professional service outlined in section 5 that is authorised by the radiologist who provided the healthcare, or on whose behalf the healthcare was provided. Subsection 6(2) specifies that images are excluded. Only information from written reports is required to be uploaded.

Report is the generally accepted term to reflect the format of information that is provided to a requesting or treating healthcare practitioner, outlining the results of a particular diagnostic service. The effect of these rules is that the information or data elements contained in that report, or other document, are to be shared to the individual's My Health Record, in accordance with the system specifications developed by the My Health Record System Operator.

The note to section 6 confirms that the information is required to be shared with the My Health Record system for medicare benefits to be payable for that service, unless an exception applies. Exceptions are outlined in section 19AD of the Act or may be specified in rules made under the MHR Act.

Section 7 - Diagnostic imaging services—period for sharing information with the My Health Record system

Section 7 specifies the time period within which the information specified in section 6 must be uploaded to My Health Record in order for medicare benefits to be payable.

Information needs to be uploaded within 24 hours of when the information is first provided to the practitioner who requested the professional service, or if there was no request, for example if the service is one the radiologist themselves determined was necessary, to the practitioner who is treating the person to whom the information relates.

The 24 hour period supports technical implementation matters, such as where reports may be sent to My Health Record in batches, or scheduled for upload outside of normal business hours. This timeframe reflects expectations that information should be shared to My Health Record at the same time as it is shared to the healthcare recipient's requesting or treating healthcare provider.

Division 2 – Pathology services

Section 8 – Pathology services—specified professional services and persons

Section 8 outlines the types of pathology services and practitioners impacted by the upload condition, subject to any additional exceptions outlined in the Act or rules made under the MHR Act.

Section 8 specifies for the purpose of subsection 19AD(1), the following services:

- a. a pathology service referred to in an item of the table in Part 2 of Schedule 1 to the Health Insurance (Pathology Services Table) Regulations 2020 or additionally determined by the Minister to be treated as if it were in that table
- b. a pathology service that due to a Ministerial determination under the Act, is treated as if it were in the Health Insurance (Pathology Services Table) Regulations 2020; or
- c. a pathologist determinable service within the meaning of the Act.

The specified services need to be rendered by or on behalf of an approved pathology practitioner within the meaning of the Act to be impacted by the upload condition.

Section 9 - Pathology services—information to be shared with the My Health Record system

Section 9 specifies the information that needs to be uploaded to the My Health Record system. It specifies for the purposes of subsection 19AD(1), the information contained in a report or other document that is created in relation to the healthcare outlined in section 8 that is authorised by the approved pathology practitioner who provided the healthcare, or on whose behalf the healthcare was provided.

Report is the generally accepted term to reflect the format of information that is provided to a requesting or treating healthcare practitioner, outlining the results of a particular pathology service. The effect of these rules is that the information or data elements contained in that report, or other document, are to be shared to the consumer's My Health Record, in accordance with the system specifications developed by the My Health Record System Operator.

The note to section 9 confirms that the information is required to be shared with the My Health Record system for medicare benefits to be payable for that service, unless an exception applies. Exceptions are outlined in section 19AD of the Act or may be specified in rules made under the MHR Act.

Section 10 - Pathology services—period for sharing information with the My Health Record system

Section 10 specifies the time period within which the information specified in section 9 must be uploaded to My Health Record in order for medicare benefits to be payable.

Information needs to be uploaded within 24 hours of when the information is first provided to the practitioner who requested the professional service, or if there was no request, for example if the service is a pathologist determinable service, the practitioner who is treating the person to whom the information relates.

The 24 hour period supports technical implementation matters, such as where reports may be sent to My Health Record in batches, or scheduled for upload outside of normal business hours. This timeframe reflects expectations that information should be shared to My Health Record at the same time as it is shared to the healthcare recipient's requesting or treating healthcare provider. This would include the requirement to upload partial results where a series of tests have been requested, but some finalised results are available and shared to a health care provider prior to all tests being completed.

Statement of Compatibility with Human Rights

Prepared in accordance with Part 3 of the Human Rights (Parliamentary Scrutiny) Act 2011

Health Insurance (Share by Default) Rules 2025

This Disallowable Legislative Instrument is compatible with the human rights and freedoms recognised or declared in the international instruments listed in section 3 of the *Human Rights (Parliamentary Scrutiny) Act 2011*.

Overview of the Disallowable Legislative Instrument

The *Health Legislation Amendment (Modernising My Health Record - Sharing by Default) Act 2025* (SBD Act) introduced reforms aimed at enhancing the accessibility and utility of health records in Australia for consumers and their care teams.

The SBD Act amended the *My Health Records Act 2012* (the MHR Act) and *Health Insurance Act 1973* (the Act) to implement a new regulatory framework to mandate the sharing of consumer health information to My Health Record. The Act was amended to make Medicare benefits for prescribed services only payable when prescribed information has been uploaded. The MHR Act was amended to create an obligation on specified constitutional corporations to become registered under the Act and to upload prescribed information. The amendments in the SBD Act ensure that where an obligation applies to either a constitutional corporation or an individual healthcare provider or both, the obligation will be satisfied if either uploads the prescribed information.

The SBD Act is intended to support consistent and streamlined health information sharing while maintaining robust privacy protections. It embeds the principles of transparency, accountability, and consumer choice and control.

The Instrument specifies certain pathology and diagnostic imaging services as subject to new upload conditions in order for medicare benefits to be payable, by reference to medicare benefit scheme tables as set out in other health insurance regulations. The Instrument outlines the information required to be uploaded are approved pathology practitioner and radiologist reports resulting from the specified pathology and diagnostic imaging services. Uploading to My Health Record will be required within 24 hours of the report being sent to a requesting or treating practitioner. Approved pathology practitioners and radiologists must arrange for uploading of the relevant information or be associated with a healthcare provider organisation that is registered for My Health Record under the MHR Act and processes the uploads.

Human rights implications

The Instrument engages the following rights:

- Article 12(1) of the International Covenant on Economic, Social and Cultural Rights (ICESCR) being the right to the enjoyment of the highest attainable standard of physical and mental health; and
- The right to privacy under Article 17 of the International Covenant on Civil and Political Rights (ICCPR).

The Right to Health

Article 12(1) of the ICESCR provides that everyone has the right to the enjoyment of the highest attainable standard of physical and mental health. To give effect to this right, better sharing of health information will support healthcare recipients and their healthcare providers to have access to information required to make informed decisions about their health and care planning. Better access to health information will also support greater accessibility of health information and services. Ensuring that healthcare recipients and their care team have timely access to their pathology and diagnostic imaging reports will reduce the need for unnecessary duplicate testing and speed up diagnosis and treatment times. It will also decrease the amount of time healthcare providers have to spend chasing up past reports.

The Strengthening Medicare Taskforce Report, released in 2022, highlighted that access to real time health information is a critical foundation for a modern and connected healthcare system. It recommended requiring that information be shared by default for private and public practitioners and services.

The share by default conditions in the Instrument will ensure consistent and timely health information sharing to support a healthcare recipient's healthcare journey across different healthcare settings.

This Instrument provides that medicare benefits can only be paid for specified services if the required information is uploaded. The ability to recoup Medicare benefits does not impact the cost of the service to the individual and no debt can be raised against the individual under these proposed rules.

The Instrument promotes the right to an adequate standard of living and the right to the enjoyment of the highest attainable standard of physical and mental health for Australians.

The Right to Privacy

The protection against arbitrary or unlawful interference with privacy is contained in Article 17 of the ICCPR. Article 17 provides that no one shall be subjected to arbitrary or unlawful interference with his or her privacy, family, home or correspondence, nor to unlawful attacks on their honour or reputation, and that everyone has the right to the protection of the law against such interference or attacks.

Although the United Nations Human Rights Committee has not defined 'privacy', it should be understood to comprise freedom from unwarranted and unreasonable intrusions into activities that society recognises as falling within the sphere of individual autonomy.

The right to privacy includes respect for informational privacy, including in respect of storing, using and sharing private information and the right to control the dissemination of personal and private information. The right to privacy also includes the right to the protection of one's personal data. The Human Rights Committee has said that, pursuant to Article 17(2) of the ICCPR, States are required to regulate the processing, use and conveyance of automated personal data, and to protect those affected against misuse. The Committee has said, moreover, that State Parties must take all appropriate measures to ensure that the gathering, storage and use of sensitive personal data is consistent with their obligations under Article 17.

The right to privacy under Article 17 can be permissibly limited in order to achieve a legitimate objective and where the limitations are lawful and not arbitrary. The term ‘unlawful’ in Article 17 of the ICCPR means that no interference can take place except as authorised under domestic law. Additionally, the term ‘arbitrary’ in Article 17(1) of the ICCPR means that any interference with privacy must be in accordance with the provisions, aims and objectives of the ICCPR and should be reasonable in the particular circumstances. The Committee has interpreted ‘reasonableness’ to mean that any limitation must be proportionate and necessary in the circumstances.

To the extent that the share by default requirements in the Instrument authorise the collection, use or disclosure of personal or health information or may interfere with the right to privacy, such measures are lawful and nonarbitrary. The measure aims to achieve the legitimate objective of assuring the consistency and completeness of key health information as well as improved access to a healthcare recipient’s own information.

The limitations on the right to privacy under the Bill are reasonable, necessary and proportionate as they appropriately balance the competing objectives of transparency and oversight with an individual’s right to privacy.

The SBD Act contains protections to ensure health information is being collected, used and disclosed in an appropriate and non-invasive manner to achieve legitimate public health aims and objectives.

The My Health Records Act and the Health Insurance Act both provide exceptions to the share by default requirements. Information cannot be uploaded to an individual’s My Health Record against their wishes as individual preference is a valid exception to the share by default requirement. Their nominated or authorised representative can also make this choice on behalf of the healthcare recipient if they are unable to make it themselves. An individual’s privacy is further safeguarded by a discretion that can be exercised by healthcare providers that sharing the individual’s health information would be detrimental to their health, safety or wellbeing.

Conclusion

The Instrument is compatible with human rights because it promotes better health outcomes for Australians by enabling efficient and effective access to health services by trusted providers. The Instrument engages the right to privacy for the legitimate objective of promoting better access to health services and to the extent that the right to privacy is limited, this is reasonable, necessary and proportionate in the circumstances.

Mark Butler
Minister for Health and Ageing